

Deep research report: quotes and evidence linking ADHD and addiction for OTOS Continuity

Executive summary

A substantial body of UK health-policy language now explicitly links untreated/unsupported ADHD with elevated risk of substance misuse and argues for care models that *avoid diagnostic silos* and improve continuity across services. The **NHS England independent ADHD Taskforce** states that unsupported ADHD is “a potent route” into outcomes including **substance misuse**, and recommends a “holistic, stepped, joined-up, generalist approach” spanning sectors because “different sectors frequently work in isolation”. The Taskforce’s Part 2 report is even more explicit about **crossing diagnostic silos**, calling for “a single, accessible front door” and stating that “ADHD care should be seamless.”

On clinical guidance, **NICE NG87** recommends improved “integration” between child health services, CAMHS and adult mental health services, plus communication protocols and transition arrangements—core continuity mechanics relevant to dual diagnosis pathway design. **Royal College of Psychiatrists** guidance for adult ADHD (CR235) explicitly states that “significant and comorbid substance misuse should be managed jointly between general adult and addiction services.”



Two web-safe, commissioner-safe anchor lines (use verbatim):

- **“ADHD, when unsupported, is a potent route into ... substance misuse...”** — NHS England Independent ADHD Taskforce (Part 1, 2025)
- **“Significant and comorbid substance misuse should be managed jointly between general adult and addiction services...”** — Royal College of Psychiatrists (CR235, 2023)

UK service-level documentation also makes the ADHD → SUD risk link explicit. For example, **Hywel Dda University Health Board** adult ADHD prescribing guidance lists “increased incidence of substance use disorder (SUD)” among consequences of untreated ADHD and anticipates “increased joint working” for patients with co-morbidities and complex needs.

On evaluated/observational pathway prototypes, an NHS addiction setting in London has published a case series showing that routine ADHD assessment within an addiction clinic identified ADHD in **15%** of patients and concluded there is a “strong association between ADHD and addiction,” recommending routine ADHD screening in club-drug users and emphasising “integrated mental health services.” Separately, a University of Cambridge / Metropolitan Police Service feasibility study screening arrestees in London (2024 data; published 2025) found that **nearly 60%** of those arrested for drug offences had an existing diagnosis or a positive ADHD screening result—supporting “early-stage” screening and diversion/“no wrong door” logic at the criminal-justice interface.

For your proposed terminology (“NIA” / *Neurologically Informed Addiction*), the strongest “bridge” language in UK policy is currently framed as **co-occurring conditions**, **no wrong door**, **joined-up working**, and **continuity of care** rather than an established “neurologically informed addiction” label. A new DHSC delivery framework for co-occurring mental health and substance use conditions explicitly mandates “No wrong door” and says services should be “in seamless partnerships” and improve “continuity of care.” In parallel, academic literature uses phrases such as “neurologically informed therapeutic interventions” and “neurologically informed treatment paths,” but this is not yet a standard UK commissioning term.

Quote bank for website and partner-portal use

The table below prioritises short, **directly verifiable** extracts with an auditable source location (page/line context where available). Where the quote comes from a transcript, it is quoted *as published* in that transcript.

**Quick pick list (lowest-risk quotes for the OTOS site)**

- 1) **Taskforce (Part 1):** "ADHD, when unsupported, is a potent route into ... substance misuse..."
- 2) **Taskforce (Part 2):** "ADHD care should be seamless."
- 3) **Taskforce (Part 2):** "There should be a single, accessible front door."
- 4) **RCPsych (CR235):** "Significant and comorbid substance misuse should be managed jointly between general adult and addiction services..."
- 5) **DHSC (Co-occurring framework):** "No wrong door: ... services should be in seamless partnerships to meet needs."

Use rule: keep these as short pull-quotes; don't paraphrase the meaning in a way that becomes a new claim.

Quote (exact wording)	Speaker	Role	Source (with section/page context)	Year	URL
"Addictions all work on the dopamine system, which is what is affected in ADHD as well."	Gabor Maté	Physician & author (as credited by the host site)	The Mel Robbins Podcast episode page transcript ("Why Pain Avoidance Creates More Pain" clip text).	2024	https://www.melrobbins.com
"The problem in addiction is not lack of free will, but lack of a free won't."	Gabor Maté	Physician & author	Same transcript section immediately preceding the dopamine/ADHD linkage (note transcript phrasing).	2024	https://www.melrobbins.com
"...doesn't develop well in people with ADHD and incidentally in people who are addicted, which is why there's such a great link between addictions and ADHD."	Gabor Maté	Physician & author	Same episode page transcript (impulse-regulation circuitry; transcript line context shows the explicit ADHD ↔ addiction link).	2024	https://www.melrobbins.com
"...if the environment is actually creating the conditions in your brain and body for addiction and ADHD..."	Mel Robbins (quoted question/statement in transcript)	Podcaster & author	Episode page transcript (environment framing; transcript attributes this line to the host, not Maté).	2024	https://www.melrobbins.com
"ADHD, when unsupported, is a potent route into ... substance misuse..."	NHS England independent ADHD Taskforce (report voice)	Independent Taskforce report	ADHD Taskforce Part 1 PDF, Abbreviated summary (p.3 in PDF pagination; early lines).	2025	https://www.england.nhs.uk/content/uploads/2025/06/PRN-independent-adhd-taskforce-
"We recommend a holistic, stepped, joined-up, generalist approach..."	NHS England independent ADHD Taskforce	Independent Taskforce report	ADHD Taskforce Part 1 PDF, Abbreviated summary recommendations section (service model).	2025	https://www.england.nhs.uk/content/uploads/2025/06/PRN-independent-adhd-taskforce-

Quote (exact wording)	Speaker	Role	Source (with section/page context)	Year	URL
"Different sectors frequently work in isolation from each other..."	NHS England independent ADHD Taskforce	Independent Taskforce report	ADHD Taskforce Part 1 PDF, Executive summary / cross-sector context.	2025	https://www.england.nhs.uk/content/uploads/2025/06/PRN-independent-adhd-taskforce-
"ADHD care should be seamless."	NHS England independent ADHD Taskforce	Independent Taskforce report	ADHD Taskforce Part 2 PDF, recommendations snapshot / summary language.	2025	https://www.england.nhs.uk/content/uploads/2025/06/PRN-task-adhd-taskforce-part-2.
"...avoid excluding children ... or reliance on diagnostic silos ... given that co-morbidity and overlaps are so common."	NHS England independent ADHD Taskforce	Independent Taskforce report	ADHD Taskforce Part 2 PDF, early intervention / school programmes section (explicit anti-silo statement).	2025	https://www.england.nhs.uk/content/uploads/2025/06/PRN-task-adhd-taskforce-part-2.
"There should be a single, accessible front door."	NHS England independent ADHD Taskforce	Independent Taskforce report	ADHD Taskforce Part 2 PDF, early sections on service design.	2025	https://www.england.nhs.uk/content/uploads/2025/06/PRN-task-adhd-taskforce-part-2.
"...expand holistic support that crosses diagnostic silos and agencies..."	NHS England independent ADHD Taskforce	Independent Taskforce report	ADHD Taskforce Part 2 PDF, Recommendation 11 (stepped care; cross-agency).	2025	https://www.england.nhs.uk/content/uploads/2025/06/PRN-task-adhd-taskforce-part-2.
"People with attention deficit hyperactivity disorder (ADHD) would benefit from ... better integration of ... CAMHS and adult mental health services."	NICE	UK clinical guideline authority	NICE NG87 recommendations, "Service organisation" (recommendation 1.1.1).	2018 (reviewed 2025)	https://www.nice.org.uk/gu
"Significant and comorbid substance misuse should be managed jointly between general adult and addiction services..."	Royal College of Psychiatrists	Professional body guidance	RCPsych Scotland adult ADHD good practice guidance (CR235 PDF).	2023	https://www.rcpsych.ac.uk/care/better-mh-policy/college-good-practice-guidance.pdf
"Increased incidence of substance use disorder (SUD)."	Hywel Dda University Health Board	NHS health board service guidance	Adult ADHD Service Specification & Prescribing Guidelines, "significant consequences of untreated ADHD" bullet list.	2025	https://hduhb.nhs.wales/abarrangements/freedom-of-information-log-appendices/1-attention-adhd-and-community-drug-and-381kb/
"It is hoped that establishing such a service"	Hywel Dda University Health Board (document voice)	NHS health board service guidance	Same document, background/service rationale (explicit	2025	https://hduhb.nhs.wales/abarrangements/freedom-of-information-log-appendices/1-attention-

Quote (exact wording)	Speaker	Role	Source (with section/page context)	Year	URL
will provide a platform for increased joint working..."			joint-working intent).		adhd-and-community-drug-and-381kb/
"GPs will be familiar with the consequences of ADHD (for example, substance misuse...)"	British Journal of General Practice article authors	Primary care clinicians/academics	BJGP clinical article on adult ADHD (open on PubMed Central).	2020	https://pmc.ncbi.nlm.nih.gov
"No wrong door: ... Treatment for co-occurring conditions is available through every contact point and services should be in seamless partnerships to meet needs."	Department of Health and Social Care (framework voice)	UK government delivery framework	Co-occurring mental health and substance use delivery framework (core principles).	2025	https://www.gov.uk/government/mental-health-and-substance-occurring-mental-health-and
"Assertive linkage to appropriate mental healthcare should be considered a core role for substance use services and joint working is to be encouraged."	Royal College of Psychiatrists (report voice)	Co-occurring policy/clinical recommendations	RCPsych CoSUM report, clinical management section (no wrong door + linkage).	2025	https://www.rcpsych.ac.uk/care/better-mh-policy/collecting-cosum.pdf
"What I learned is that the opposite of addiction is not sobriety... The opposite of addiction is human connection."	Johann Hari	Author/journalist	Interview in The Guardian.	2015	https://www.theguardian.com/interview-drugs-book-independence
"To ensure fair treatment in the criminal justice system, we need to understand how neurodivergence affects interactions with the law"	Tanya Procyshyn	Cambridge researcher (as quoted)	University of Cambridge news release (quoteblock).	2025	https://www.cam.ac.uk/research-news/in-london-may-have-undiagnosed-adhd
"Early identification helps officers interpret behaviours ..."	Dion Brown	Police / study co-author (as quoted)	Cambridge Department of Psychiatry news release (quoteblock).	2025	https://www.psychiatry.cam.ac.uk/news/london-may-have-undiagnosed-adhd

Quote (exact wording)	Speaker	Role	Source (with section/page context)	Year	URL
and ensures appropriate support is provided."					
"We must ensure that 'No Wrong Door' means that every door is open."	Mental Welfare Commission for Scotland (report voice)	Public-service oversight report	"Ending the Exclusion" report on co-occurring mental health and problem substance use.	2022	https://www.mwscot.org.uk/09/EndingTheExclusion_Septe
"In 2032, there will be no wrong door for anyone wanting support for their mental health or neurodiverse needs."	Centre for Mental Health / NHS Confederation (report voice)	System-vision document	"No wrong door" vision report (access section).	2022	https://www.nhsconfed.org/door_vision-for-MH-learning-in-2032.pdf

How to use this page (fast)

Use this as a *source vault* to feed partner pages and decks without inventing new claims.

1. **Pick 1 policy quote + 1 practice quote** (e.g., Taskforce + RCPsych).
2. Pair with one plain English OTOS line:
 - "Handoff receipts — so you can see whether the next step actually happened."
 - "Drift signals — so you see silent disengagement before it becomes crisis."
3. Keep every page claims-safe:
 - Quote the source verbatim.
 - Add **TODO**: if you don't have a citation for a number.
 - Avoid "first-of-its-kind" language.

Notes on quote verification and gaps

The UK has a highly relevant practitioner article—"Bridging the gap: Reflections on the development and implementation of an ADHD assessment pathway in a community drug and alcohol service"—but I was not able to access its full text through the available retrieval interface (source fetch errors). The existence, authorship and summary description are still verifiable via Change Grow Live's research listing and the citation/DOI, but detailed methods/outcomes remain **unspecified** here.

UK policy and guidance connecting ADHD with addiction risk and service continuity

The policy thread that best supports OTOS Continuity's "awareness layer" framing is that UK system leaders increasingly acknowledge (a) **co-morbidity/overlap is common** and (b) siloed pathways create harm and inefficiency, so the system needs continuity mechanisms and "single front door" behaviours.

The NHS England ADHD Taskforce report language is unusually direct for a national document. It explicitly links unsupported ADHD to "substance misuse" in the short abbreviated summary, alongside crime, suicide and other outcomes, and then argues for a cross-sector response because ADHD "is not the remit of health alone." It also explicitly recognises silo dynamics: "Different sectors frequently work in isolation from each other" and calls for cross-agency working centred "on the whole person." Part 2 carries the same thread into service design rules: "There should be a single, accessible front door," and interventions must "avoid... reliance on diagnostic silos... given that co-morbidity and overlaps are so common." The report also uses unusually strong continuity wording: "ADHD care should be seamless."

NICE NG87 gives you a stable UK-standard line for integration and continuity even when the immediate focus is not addictions: it states that people with ADHD would benefit from better integration across child health services, CAMHS and adult mental health services, and recommends communication protocols, information sharing and transition arrangements. This is highly reusable language for "continuity" claims in NHS-facing materials because it is normative guidance rather than opinion.

On explicit ADHD ↔ addiction pathway design, **RCPsych adult ADHD good practice guidance (CR235)** gives a clean, NHS-usable sentence: "Significant and comorbid substance misuse should be managed jointly between general adult and addiction services...". In practical terms, this is a direct endorsement of a dual-diagnosis/CoSUM-style joint working model applied to ADHD.

At NHS trust/service-specification level, **Hywel Dda University Health Board's Adult ADHD Service Specification and Prescribing Guidelines** provides two particularly valuable elements for your portal demonstration: (i) a straightforward declaration that untreated ADHD is associated with "increased incidence of substance use disorder (SUD)", and (ii) operational pathway hooks, including stabilisation expectations (e.g., problematic/unstable substance use considerations) and explicit diversion/misuse risk assessment in ADHD medication processes. Those operational details are directly relevant to OTOS Continuity's proposition that "ADHD + addiction" is a predictable intersection that should be handled *systemically*, not as an exception case.

For the broader "no wrong door / continuity" narrative that underpins your "connect the silos" goal, DHSC's co-occurring conditions delivery framework (2025) is among the clearest current UK-government statements: it defines "No wrong door" and specifies that "treatment... is available through every contact point and services should be in seamless partnerships to meet needs," and it explicitly calls for "better continuity of care" between drug and alcohol and mental health services. While the framework is not ADHD-specific, it is strategically useful because ADHD+addiction presentations often fall within (or adjacent to) co-occurring mental health/substance use pathways in practice, and the framework language supports exactly the "avoid silos" argument OTOS Continuity wants to foreground.

UK trials, pilots and service evaluations relevant to integrated ADHD–addiction pathways

The strongest UK "bridge" evidence (in terms of evaluated real-world models, not just principles) currently clusters in three places: (1) addiction services adopting ADHD screening/diagnosis, (2) justice interfaces developing neurodivergence screening as an early diversion/continuity mechanism, and (3) no-wrong-door integrated models for complex needs that prevent people being passed between services.

Trials/pilots/service evaluations table

Name	Lead organisation(s)	Dates	Design	Key outcomes reported	URL
ADHD assessment and treatment within the Club Drug Clinic (case series)	Central and North West London NHS Foundation Trust (Club Drug Clinic)	Service cohort "since July 2023"; published June 2025	Case series / service evaluation (conference abstracts; open access)	15 out of 98 patients (15%) were diagnosed with ADHD and commenced on atomoxetine; conclusion states "strong association between ADHD and addiction" and recommends routine ADHD screening in club drug users; emphasises importance of integrated mental health services.	https://www.cambridge.org/core/services/a/core/content/view/F5A41B37F7521378272508F1case-series.pdf

Name	Lead organisation(s)	Dates	Design	Key outcomes reported	URL
Neurodiversity in Custody screening study (ADHD/autistic traits in arrestees)	University of Cambridge + Metropolitan Police Service	8-week screening period in 2024; published 2025	Feasibility study / cross-sectional screening in custody suites	Of 303 eligible arrestees, 216 (71.3%) consented; screening suggested 50% warranted further ADHD assessment; "nearly 60%" of those arrested for drug offences had an existing diagnosis or positive ADHD screening result—supporting early identification and referral pathways.	https://pmc.ncbi.nlm.nih.gov/articles/PMC
No Wrong Door Innovation Programme evaluation	North Yorkshire County Council (Innovation Programme) / DfE (evaluation publication)	Hubs set up April 2015; evaluation covers April 2015–Sept 2016	Mixed-methods programme evaluation (integrated hub + outreach model)	Evaluation describes an "integrated service" and says the team supports young people "throughout their journey" so they are "not passed from service to service"; quantitative outcomes include 290 supported; ~77% referred "edging to, or on the edge of, care"; 86% of that group remained out of care with support; among those already looked after at referral, 40% ceased being looked after.	https://assets.publishing.service.gov.uk/

How these pilots map to OTOS Continuity's "connect the silos" thesis

The **Club Drug Clinic case series** demonstrates something very close to what OTOS Continuity is arguing for: an addiction service that does not treat ADHD as "somebody else's diagnosis", but instead identifies it, treats it, and recommends routine screening in its population. This is a practical prototype for "NIA" in the sense that it operationalises neurodevelopmental assessment within addiction work.

The **custody-suite screening study** is not an addiction-treatment trial, but it is a strong continuity argument for NHS/Cambridge stakeholders: it suggests that a large fraction of drug-offence arrestees show ADHD traits and would

benefit from earlier identification and referral, which is essentially an upstream “no wrong door” intervention that could prevent repeated crisis/justice contact.

The **No Wrong Door** evaluation provides a language template for why continuity matters: the integrated team exists specifically so people are not “passed from service to service.” That phrase aligns almost perfectly with OTOS Continuity’s “awareness layer” value proposition.

Synthesis for proposing “NIA” and ready-to-use attributions

What the evidence supports

Across UK sources, there is convergent support for three propositions that can underpin a “Neurologically Informed Addiction (NIA)” framing without overclaiming:

First, ADHD and addiction share plausible neurocognitive mechanisms and high observed overlap. Gabor Maté’s interview framing explicitly links ADHD and addiction through dopamine and impulse-control circuitry. National policy likewise treats ADHD as a route into substance misuse when unsupported.

Second, siloed service structures are named as a problem at the highest UK levels. The ADHD Taskforce warns against “diagnostic silos” and calls for a “single... front door” and “seamless” ADHD care. DHSC openly states that co-occurring pathways require “seamless partnerships” and better “continuity of care.”

Third, there are feasible, UK-based prototypes of integrated detection and management. The Club Drug Clinic experience provides a concrete “addiction service + ADHD assessment/treatment” case series and explicitly recommends routine screening to reduce longer-term misuse.

What is *not* yet established (and how to phrase it safely)

I did not find UK commissioning or guideline documents that formally define a term like “Neurologically Informed Addiction” as a recognised category; the established policy terms are **dual diagnosis / co-occurring conditions / no wrong door / trauma-informed care**.

However, “neurologically informed” language does exist in the research literature discussing addiction treatment and neurobiology (e.g., “neurologically informed therapeutic interventions,” “neurologically informed treatment paths”). This gives you a defensible way to present NIA as a **proposed synthesis term** rather than a pre-existing standard.

Recommended one-line attributions for your website/portal

These are designed to be copy-pasteable and NHS-facing (clear provenance, minimal interpretation):

“Addictions all work on the dopamine system, which is what is affected in ADHD as well.” — Gabor Maté, *The Mel Robbins Podcast*, Episode 235 (21 Nov 2024).

“ADHD, when unsupported, is a potent route into... substance misuse...” — NHS England Independent ADHD Taskforce, *Report Part 1* (published 20 Jun 2025).



“ADHD care should be seamless.” — NHS England Independent ADHD Taskforce, *Report Part 2* (published 6 Nov 2025).

“Significant and comorbid substance misuse should be managed jointly...” — Royal College of Psychiatrists, *ADHD in Adults: Good practice guidance (CR235)* (2023).

"Treatment... should be in seamless partnerships to meet needs." — DHSC, *Co-occurring mental health and substance use delivery framework* (10 Dec 2025).

"This series reflects the strong association between ADHD and addiction..." — CNWL / *BJPsych Open* abstract, "ADHD and Substance Misuse in the Club Drug Clinic" (published online 20 Jun 2025).

"To ensure fair treatment... we need to understand how neurodivergence affects interactions with the law." — Tanya Procyshyn, University of Cambridge news release on custody screening study (10 Dec 2025).

"The opposite of addiction is human connection." — Johann Hari, *The Guardian* interview (2 Jan 2015).

Visualisation ideas and suggested assets for the portal

A partner-portal demonstration usually lands better when the conceptual case ("stop silos") is immediately grounded in a small number of "anchor artefacts" (policy + real-world prototype + measurable outcome). Below is a timeline chart idea and a small set of visual concepts that map directly to the evidence above.

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2015Johann Hari"opposite of addiction is...connection"(Guardian)2018NICE NG87recommends betterintegration acrossADHD care pathways2015-2016No Wrong Door hubspiloted; integratedteam "not passedfrom service toservice"2024London custodyscreening studyperiod (ADHD traits +drug offences link)2025-06NHS EnglandIndependent ADHDTaskforce Part 1published (ADHD +substance misuse;joined-up care)2025-06Club Drug Clinic caseseries published(screening/treatingADHD insideaddiction service)2025-11NHS EnglandIndependent ADHDTaskforce Part 2published (singlefront door; avoiddiagnostic silos)2025-12DHSC Co-occurringframework published(No wrong door;seamlesspartnerships;continuity)UK policy + prototype signals connecting ADHD, addiction and "no wrong door"
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Show code

Concept diagrams that fit NHS/Cambridge audiences (and are low-risk to visualise accurately):

A "Single Front Door" service map: Inputs (GP, addiction services, custody, A&E) → shared triage → stepped care (self-management, ADHD assessment, addiction treatment, mental health support) aligned to Taskforce language.

A "Silo failure loop" diagram: ADHD under-recognition → dysregulation/impulsivity → self-medication/substance misuse → justice/health crises → fragmented referrals → missed ADHD follow-up, mapped to Taskforce and custody-study findings.

A clinic-integration example card: "Addiction service embeds ADHD screening & treatment" (Club Drug Clinic case series) as a concrete "it can be done" UK example.